



House of Representatives

General Assembly

File No. 539

February Session, 2026

House Bill No. 5513

House of Representatives, April 9, 2026

The Committee on Public Health reported through REP. MCCARTHY VAHEY of the 133rd Dist., Chairperson of the Committee on the part of the House, that the bill ought to pass.

***AN ACT CONCERNING THE DEPARTMENT OF PUBLIC HEALTH'S
RECOMMENDATIONS REGARDING VARIOUS REVISIONS TO THE
PUBLIC HEALTH STATUTES.***

Be it enacted by the Senate and House of Representatives in General Assembly convened:

- 1 Section 1. (NEW) (*Effective from passage*) The Commissioner of Public
- 2 Health may appoint a willing licensed embalmer, as defined in section
- 3 20-207 of the general statutes, to dispose of an abandoned dead human
- 4 body or the abandoned cremated remains of a dead human body. An
- 5 appointed embalmer may request any records necessary to identify
- 6 such dead human body or cremated remains. The appointed embalmer
- 7 shall notify the commissioner, in a form and manner prescribed by the
- 8 commissioner, of the date and time of disposition of any such dead
- 9 human body or cremated remains and the method of such disposition
- 10 not later than seven days after such disposition. A dead human body or
- 11 cremated remains of a dead human body shall be deemed to be
- 12 abandoned, for the purposes of this section, when a licensed or formerly
- 13 licensed funeral director or embalmer voluntarily or improperly
- 14 relinquishes custody of such human body or cremated remains in

15 violation of the requirements of state law, including, but not limited to,
16 when a funeral director or embalmer ceases to operate a funeral home
17 and fails to arrange for the transfer or disposition of such dead human
18 body or cremated remains in the funeral director or embalmer's
19 possession prior to ceasing operations.

20 Sec. 2. Subsections (c) and (d) of section 7-51a of the general statutes
21 are repealed and the following is substituted in lieu thereof (*Effective*
22 *from passage*):

23 (c) [For deaths occurring on or after July 1, 1997, the] The Social
24 Security number recorded on the death certificate of the deceased
25 person shall be [recorded in the "administrative purposes" section of the
26 death certificate. Such administrative purposes section, and the Social
27 Security number contained therein, shall be] restricted and disclosed
28 only to the following eligible parties: (1) All parties specified on the
29 death certificate, including the informant, licensed funeral director,
30 licensed embalmer, conservator, surviving spouse, physician or
31 advanced practice registered nurse and town clerk, for the purpose of
32 processing the certificate, (2) the surviving spouse, (3) the next of kin, or
33 (4) any state and federal agencies authorized by federal law to receive
34 the Social Security number. The department shall provide any other
35 individual, researcher or state or federal agency requesting a certified or
36 uncertified copy of a death certificate, [or the information contained
37 within such certificate, for a death occurring on or after July 1, 1997, such
38 certificate or information. The] in accordance with the provisions of this
39 section, such copy with the decedent's Social Security number [shall be]
40 removed or redacted. [from such certificate or information or the
41 administrative purposes section shall be omitted from such certificate.]

42 (d) The department shall provide, as electronic data, the information
43 contained within a certified copy of a death certificate to any individual,
44 researcher or state or federal agency upon request. The decedent's Social
45 Security number shall be removed, redacted or omitted from such data
46 unless the requester is a state or federal agency authorized by federal
47 law to receive the Social Security number.

48 [(d)] (e) The registrar of vital statistics of any town or city in this state
49 that has access to an electronic vital records system, as authorized by the
50 department, may use such system to issue certified copies of birth,
51 death, fetal death or marriage certificates that are electronically filed in
52 such system.

53 Sec. 3. Subsection (a) of section 19a-537 of the general statutes is
54 repealed and the following is substituted in lieu thereof (*Effective from*
55 *passage*):

56 (a) As used in this section and section 19a-537a:

57 (1) "Vacancy" means a bed that is available for an admission;

58 (2) "Nursing home" means any chronic and convalescent facility or
59 any rest home with nursing supervision, as defined in section 19a-521;

60 (3) "Hospital" means a general short-term hospital licensed by the
61 Department of Public Health or a hospital for [mental illness]
62 psychiatric disabilities, as defined in section 17a-495, or a chronic
63 disease hospital.

64 Sec. 4. Section 19a-36g of the general statutes is repealed and the
65 following is substituted in lieu thereof (*Effective from passage*):

66 As used in this section, [and] sections 19a-36b, 19a-36h to 19a-36o,
67 inclusive, as amended by this act, and section 19a-36r:

68 (1) "Catering food service establishment" means a business that is
69 involved in the (A) sale or distribution of food and drink prepared in
70 bulk in one geographic location for retail service in individual portions
71 in another location, or (B) preparation and service of food in a public or
72 private venue that is not under the ownership or control of the operator
73 of such business;

74 (2) "Certified food protection manager" means a food employee that
75 has supervisory and management responsibility and the authority to
76 direct and control food preparation and service;

77 (3) "Class 1 food establishment" means a retail food establishment
78 that does not serve a population that is highly susceptible to food borne
79 illnesses and only offers (A) commercially packaged food in its original
80 commercial package that is time or temperature controlled for safety, or
81 (B) commercially prepackaged, precooked food that is time or
82 temperature controlled for safety and heated, hot held and served in its
83 original commercial package not later than four hours after heating, or
84 (C) food prepared in the establishment that is not time or temperature
85 controlled for safety;

86 (4) "Class 2 food establishment" means a retail food establishment
87 that does not serve a population that is highly susceptible to food-borne
88 illnesses and offers a limited menu of food that is prepared or cooked
89 and served immediately, or that prepares or cooks food that is time or
90 temperature controlled for safety and may require hot or cold holding,
91 but that does not involve cooling;

92 (5) "Class 3 food establishment" means a retail food establishment
93 that (A) does not serve a population that is highly susceptible to food-
94 borne illnesses, and (B) offers food that is time or temperature controlled
95 for safety and requires complex preparation, including, but not limited
96 to, handling of raw ingredients, cooking, cooling and reheating for hot
97 holding;

98 (6) "Class 4 food establishment" means a retail food establishment
99 that serves a population that is highly susceptible to food-borne
100 illnesses, including, but not limited to, preschool students, hospital
101 patients and nursing home patients or residents, or that conducts
102 specialized food processes, including, but not limited to, smoking,
103 curing or reduced oxygen packaging for the purposes of extending the
104 shelf life of the food;

105 (7) "Cold holding" means maintained at a temperature of forty-one
106 degrees Fahrenheit or below;

107 (8) "Commissioner" means the Commissioner of Public Health or the
108 commissioner's designee;

109 (9) "Contact hour" means a minimum of fifty minutes of a training
110 activity;

111 (10) "Department" means the Department of Public Health;

112 (11) "Director of health" means the director of a local health
113 department or district health department appointed pursuant to section
114 19a-200 or 19a-242;

115 (12) "Food code" means the food code administered under section
116 19a-36h, as amended by this act;

117 (13) "Food establishment" means an operation that (A) stores,
118 prepares, packages, serves, vends directly to the consumer or otherwise
119 provides food for human consumption, including, but not limited to, a
120 restaurant, catering food service establishment, food service
121 establishment, temporary food service establishment, itinerant food
122 vending establishment, market, conveyance used to transport people,
123 institution or food bank, or (B) relinquishes possession of food to a
124 consumer directly, or indirectly through a delivery service, including,
125 but not limited to, home delivery of grocery orders or restaurant takeout
126 orders or a delivery service that is provided by common carriers. "Food
127 establishment" does not include a vending machine, as defined in
128 section 21a-34, a private residential dwelling in which food is prepared
129 under section 21a-62a or a food manufacturing establishment, as
130 defined in section 21a-151;

131 (14) "Food inspector" means a director of health, or his or her
132 authorized agent, or a registered environmental health specialist who
133 has been certified as a food inspector by the commissioner;

134 (15) "Food inspection training officer" means a certified food
135 inspector who has received training developed or approved by the
136 commissioner and been authorized by the commissioner to train
137 candidates for food inspector certification;

138 (16) "Food-borne illness" means illness, including, but not limited to,
139 illness due to heavy metal intoxications, staphylococcal food poisoning,

140 botulism, salmonellosis, shigellosis, *Clostridium perfringens*
141 intoxication and hepatitis A, acquired through the ingestion of a
142 common-source food or water contaminated with a chemical, infectious
143 agent or the toxic products of a chemical or infectious agent;

144 (17) "Food-borne outbreak" means illness, including, but not limited
145 to, illness due to heavy metal intoxications, staphylococcal food
146 poisoning, botulism, salmonellosis, shigellosis, *Clostridium perfringens*
147 intoxication and hepatitis A, in two or more individuals, acquired
148 through the ingestion of common-source food or water contaminated
149 with a chemical, infectious agent or the toxic products of a chemical or
150 infectious agent;

151 (18) "Hot holding" means maintained at a temperature of one
152 hundred thirty-five degrees Fahrenheit or above;

153 (19) "Itinerant food vending establishment" means a vehicle-
154 mounted, self-contained, mobile food establishment;

155 (20) "Permit" means a written document issued by a director of health
156 that authorizes a person to operate a food establishment;

157 (21) "Temporary food service establishment" means a food
158 establishment that operates for a period of not more than fourteen
159 consecutive days in conjunction with a single event or celebration;

160 (22) "Time or temperature controlled for safety" means maintained at
161 a certain temperature or maintained for a certain length of time, or both,
162 to prevent microbial growth and toxin production; and

163 (23) "Variance" means a written document issued by the
164 commissioner that authorizes a modification or waiver of one or more
165 requirements of the food code.

166 Sec. 5. Subsection (b) of section 19a-36h of the 2026 supplement to the
167 general statutes is repealed and the following is substituted in lieu
168 thereof (*Effective from passage*):

169 (b) The commissioner may adopt regulations, in accordance with the
170 provisions of chapter 54, to implement the provisions of this section,
171 [and] sections 19a-36i to 19a-36m, inclusive, and section 19a-36r.

172 Sec. 6. Section 19a-36b of the general statutes is repealed and the
173 following is substituted in lieu thereof (*Effective from passage*):

174 (a) Any person who serves meals to individuals at registered
175 congregate meal sites funded under Title III of the Older Americans Act
176 of 1965, as amended from time to time, which were prepared under the
177 supervision of a [qualified food operator] certified food protection
178 manager, shall be exempt from the examination requirement for
179 [qualified food operators] certified food protection managers.

180 (b) Any volunteer who serves meals for a nonprofit organization shall
181 be exempt from the examination requirement for [qualified food
182 operators] certified food protection managers.

183 (c) The Commissioner of Public Health, in conjunction with the
184 Commissioner of Social Services, shall adopt regulations in accordance
185 with the provisions of chapter 54 to establish training procedures for
186 persons exempt from the examination requirement for [qualified food
187 operators] certified food protection managers under the provisions of
188 subsections (a) and (b) of this section.

189 Sec. 7. Subsections (c) to (e), inclusive, of section 19a-580h of the
190 general statutes are repealed and the following is substituted in lieu
191 thereof (*Effective from passage*):

192 (c) Notwithstanding the provisions of sections 19a-495 and 19a-580d
193 and the regulations adopted thereunder, the Commissioner of Public
194 Health shall adopt regulations, in accordance with the provisions of
195 chapter 54, for the program established in accordance with this section
196 to ensure that: (1) Medical orders for life-sustaining treatment are
197 transferrable among, and recognized by, various types of health care
198 institutions subject to any limitations set forth in federal law; (2) any
199 procedures and forms developed for recording medical orders for life-

200 sustaining treatment require the signature of the patient or the patient's
201 legally authorized representative on the medical order for life-
202 sustaining treatment and the patient or the patient's legally authorized
203 representative is given [the original order immediately after signing
204 such] a copy of or access to the order and a copy of such order is
205 immediately placed in the patient's medical record; (3) prior to
206 requesting the signature of the patient or the patient's legally authorized
207 representative on such order, the physician, advanced practice
208 registered nurse or physician assistant writing the medical order
209 discusses with the patient or the patient's legally authorized
210 representative the patient's goals for care and treatment and the benefits
211 and risks of various methods for documenting the patient's wishes for
212 end-of-life treatment, including medical orders for life-sustaining
213 treatment; and (4) each physician, advanced practice registered nurse or
214 physician assistant that intends to write a medical order for life-
215 sustaining treatment receives training concerning: (A) The importance
216 of talking with patients about their personal treatment goals; (B)
217 methods for presenting choices for end-of-life care that elicit
218 information concerning patients' preferences and respects those
219 preferences without directing patients toward a particular option for
220 end-of-life care; (C) the importance of fully informing patients about the
221 benefits and risks of an immediately effective medical order for life-
222 sustaining treatment; (D) awareness of factors that may affect the use of
223 medical orders for life-sustaining treatment, including, but not limited
224 to, advanced health care directives, race, ethnicity, age, gender,
225 socioeconomic position, immigrant status, sexual minority status,
226 language, disability, homelessness, mental illness and geographic area
227 of residence; and (E) procedures for properly completing and
228 effectuating medical orders for life-sustaining treatment.

229 (d) A medical order for life-sustaining treatment shall not be valid
230 unless it is completed on a form prescribed by the department and
231 executed in accordance with the requirements of this section and any
232 regulations adopted under this section.

233 [(d)] (e) Nothing in this section shall be construed to limit the

234 authority of the Commissioner of Developmental Services under
 235 subsection (g) of section 17a-238 concerning orders applied to persons
 236 receiving services under the direction of said commissioner.

237 ~~[(e)]~~ (f) The Commissioner of Public Health may implement policies
 238 and procedures necessary to administer the provisions of this section
 239 until such time as regulations are adopted pursuant to subsection (c) of
 240 this section.

241 Sec. 8. Subsection (a) of section 19a-112h of the general statutes is
 242 repealed and the following is substituted in lieu thereof (*Effective from*
 243 *passage*):

244 (a) The Commissioner of Public Health shall establish and contract
 245 for the administration of a state-wide human immunodeficiency virus
 246 pre-exposure prophylaxis and post-exposure prophylaxis drug
 247 assistance program using appropriated AIDS Services funding,
 248 provided such funding is equal to or greater than twenty-five thousand
 249 dollars annually. The program shall provide financial assistance to
 250 individuals at risk of acquiring human immunodeficiency for (1) the
 251 purchase of pre-exposure and post-exposure prophylaxis for human
 252 immunodeficiency virus prescribed by a licensed physician consistent
 253 with the recommendations of the National Centers for Disease Control
 254 and Prevention, and (2) payment of associated laboratory testing and
 255 other related costs. For the purposes of this subsection, "financial
 256 assistance" includes, but need not be limited to, payments for out-of-
 257 pocket costs, copayments, coinsurance, and up to full cost payments
 258 toward a deductible for individuals who are underinsured and for
 259 whom the program is the payer of last resort.

This act shall take effect as follows and shall amend the following sections:

Section 1	<i>from passage</i>	New section
Sec. 2	<i>from passage</i>	7-51a(c) and (d)
Sec. 3	<i>from passage</i>	19a-537(a)
Sec. 4	<i>from passage</i>	19a-36g
Sec. 5	<i>from passage</i>	19a-36h(b)

Sec. 6	<i>from passage</i>	19a-36b
Sec. 7	<i>from passage</i>	19a-580h(c) to (e)
Sec. 8	<i>from passage</i>	19a-112h(a)

PH *Joint Favorable*

The following Fiscal Impact Statement and Bill Analysis are prepared for the benefit of the members of the General Assembly, solely for purposes of information, summarization and explanation and do not represent the intent of the General Assembly or either chamber thereof for any purpose. In general, fiscal impacts are based upon a variety of informational sources, including the analyst's professional knowledge. Whenever applicable, agency data is consulted as part of the analysis, however final products do not necessarily reflect an assessment from any specific department.

OFA Fiscal Note

State Impact:

Agency Affected	Fund-Effect	FY 26 \$	FY 27 \$
Public Health, Dept.	GF - Cost	See Below	See Below

Note: GF=General Fund

Municipal Impact: None

Explanation

The bill expands how AIDS Services funding may be used, resulting in a General Fund cost beginning in FY 26 as described below. The cost will reduce the AIDS Services account's recurring lapse. The other provisions of the bill have no fiscal impact.

Section 8 allows the Department of Public Health's (DPH's) contracted program for HIV pre- and post-exposure prophylaxis (PrEP and PEP) drug assistance to also provide funding for associated lab testing and related costs, resulting in a cost to DPH beginning in FY 26. The expansion of eligible program expenditures will increase spending in the AIDS Services account. The account has lapsed an average of approximately \$1.2 million annually over the past three fiscal years, and additional expenditures as allowed by the bill will decrease the amount of funding that returns to the General Fund.

The Out Years

The annualized ongoing fiscal impact identified above would continue into the future subject to inflation.

OLR Bill Analysis**HB 5513*****AN ACT CONCERNING THE DEPARTMENT OF PUBLIC HEALTH'S RECOMMENDATIONS REGARDING VARIOUS REVISIONS TO THE PUBLIC HEALTH STATUTES.*****SUMMARY**

This bill makes various changes in Department of Public Health (DPH)-related statutes and programs. It:

1. allows DPH to appoint an embalmer to dispose of an abandoned dead body or abandoned cremated remains after a funeral director or embalmer has voluntarily or improperly relinquished control of the body or remains in violation of state law (§ 1);
2. requires DPH, upon request, to issue death records electronically regardless of when the death occurred (with the Social Security number (SSN) generally redacted), instead of only just for deaths since July 1, 1997, as under current law (§ 2);
3. makes minor and clarifying changes to laws related to the food code, such as updating terminology and specifying that the DPH commissioner's authority to adopt regulations to implement food code-related laws includes a law on audits of local health department food protection programs (§§ 4-6);
4. requires that regulations for the Medical Orders for Life-Sustaining Treatment (MOLST) program allow providers to give patients a signed copy of the order rather than the original, and provides that a MOLST is invalid unless it is completed on a DPH form and executed in line with statutory and regulatory requirements (see BACKGROUND) (§ 7); and
5. allows DPH's contracted program for HIV pre- and post-

exposure prophylaxis (PrEP and PEP) drug assistance to also provide funding for associated lab testing and related costs (§ 8).

It also makes other minor, technical, and conforming changes.

EFFECTIVE DATE: Upon passage

§ 1 — DISPOSAL OF ABANDONED BODIES OR REMAINS

The bill allows the DPH commissioner to appoint a willing licensed embalmer to dispose of an abandoned dead body or abandoned cremated remains. The embalmer may request any records needed to identify the body or remains and must notify the commissioner, within seven days after the disposition of the body or remains, of when and how it was done.

Under the bill, a body or remains is deemed abandoned when a licensed or formerly licensed funeral director or embalmer voluntarily or improperly relinquishes custody in violation of the law's requirements. This includes when they stop operating a funeral home and do not provide for the transfer or disposition of the bodies or remains they had custody of at the time.

Existing law sets various requirements for funeral directors and embalmers as to the disposition of dead bodies and cremated remains, including the requirement to carry out the person's documented wishes or, in cases where there is a dispute as to who has the right to custody over the remains, the probate court's final decision.

§ 2 — ELECTRONIC DEATH RECORDS

Current law sets various requirements for death certificates, including that (1) for deaths on or after July 1, 1997, the SSN be included in an "administrative purposes" section of the certificate that can be disclosed only to certain eligible parties (such as the funeral director and surviving spouse) and (2) DPH disclose the record or (for deaths since that date) information in it upon the request of any other person, researcher, or state or federal agency, with the SSN redacted.

The bill removes the 1997 start date from these provisions and specifically requires DPH, for deaths that occurred at any time, to give these other parties information from the death certificate in electronic form. It requires the SSN to be redacted, removed, or omitted unless the requester is a state or federal agency authorized by federal law to receive it.

The bill also makes related minor and technical changes.

BACKGROUND

MOLST Program

By law, a MOLST is a medical order to carry out a patient's request for life-sustaining treatment when the patient is approaching the end stage of a serious, life-limiting illness or is in a condition of advanced, chronic progressive frailty. Patient participation is voluntary. To agree to participate, a patient or the patient's legally authorized representative must sign the MOLST form.

COMMITTEE ACTION

Public Health Committee

Joint Favorable

Yea 22 Nay 10 (03/23/2026)